Rural Health Transformation Program

State-by-State Distribution & Implementation Analysis

SLG Strategy Team

Updated April 26, 2026

1. Executive Summary

The Rural Health Transformation Program (RHTP) is a $50 billion federal program authorized under Public Law 119-21 (enacted July 4, 2025), often referenced as the One Big Beautiful Bill Act (OBBBA). The program allocates $10 billion per year across fiscal years 2026 through 2030 to the 50 U.S. states to strengthen rural healthcare delivery, workforce, infrastructure, and technology.

On December 29, 2025, the Centers for Medicare & Medicaid Services (CMS) — through the newly established Office of Rural Health Transformation (ORHT) within the Center for Medicaid and CHIP Services — announced that all 50 states received Year-1 awards. First-year awards average approximately $200 million, with a range of $147 million (New Jersey) to $281 million (Texas). The District of Columbia and U.S. territories are not statutorily eligible.

Key characteristics relevant to Dell:

- RHTP is a **cooperative agreement**, not a block grant. CMS retains ongoing oversight authority over how funds are used, and states must submit revised budgets reflecting actual award amounts for CMS approval.
- Funds flow **from CMS to states**, not directly to providers. Each state has significant discretion over sub-award mechanisms, creating 50 distinct procurement environments.
- **Technology modernization** is one of five statutorily approved use categories. Nearly every state application identified technology investment as a priority, though the specificity and scale of technology commitments varies substantially.
- RHTP funding is **not a direct offset** for projected Medicaid reductions under P.L. 119-21. CMS and multiple state agencies have explicitly stated that RHTP funds cannot be used to supplant Medicaid revenue losses. KFF estimates the reconciliation law reduces federal Medicaid spending by approximately $911 billion over ten years, with roughly $137 billion in rural areas — substantially larger than the $50 billion RHTP allocation.

Purpose of this document: Provide Dell internal stakeholders with a neutral, factual reference on RHTP structure, state-level distribution patterns, and the technology categories state plans have prioritized. This document is not a sales plan; it is a market-context briefing to support informed planning by account teams, product planners, and public sector strategy.

2. Program Structure and Funding Architecture

2.1 Legislative and Administrative Background

Element	Detail
Authorizing statute	P.L. 119-21, Section 71401 (July 2025)
Program size	$50 billion over 5 years ($10B per fiscal year)
Years covered	FY2026 through FY2030
Administering agency	CMS, Office of Rural Health Transformation (ORHT), within the Center for Medicaid and CHIP Services
Instrument type	Cooperative agreement (not a block grant)
Eligible recipients	50 U.S. states (DC and territories excluded)
Application period	Opened mid-September 2025; closed November 5, 2025
Award announcement	December 29, 2025
First-year obligation deadline	September 30, 2027

2.2 Allocation Formula

The $50 billion is split evenly between two allocation buckets. It is important to note that the second bucket is not a pure need-based formula, which was a subject of commentary from independent analysts.

Bucket A — Baseline (50% / $25 billion over 5 years): Distributed equally among all approved states. Each state receives roughly the same baseline amount regardless of size or rural population. This produces high per-capita funding for small states (Rhode Island, Delaware) and low per-capita funding for large states (Texas, California).

Bucket B — Formula / Discretionary (50% / $25 billion over 5 years): Allocated based on a set of CMS-scored factors. Per CMS's Notice of Funding Opportunity, these factors include state metrics on rurality (rural population, number of rural facilities, land area, share of DSH hospitals), current and proposed state policy actions, and the potential impact of proposed activities. According to KFF's analysis, only approximately 25% of the total fund is distributed exclusively based on measures of state rural need, and only about 5% is tied specifically to rural population. The remainder reflects policy alignment and application scoring — a point worth noting when interpreting state-level award sizes.

2.3 Per-Rural-Resident Variation

The split has produced dramatic variation in per-rural-resident funding. Examples from KFF:

State	FY2026 Total	Per-Rural-Resident (FY2026)
Rhode Island	$156.2M	~$6,305
New Jersey	$147.3M	~$1,068
Alaska	$272.2M	~$989
Texas	$281.3M	~$66
Ohio	$202.0M	~$69
North Carolina	$213.0M	~$97

2.4 Approved Uses of Funds

States were required to select at least three of the following statutorily approved use categories in their applications:

1. Evidence-based prevention and chronic disease management
2. Payments to providers for items or services, as specified by the Administrator
3. Consumer-facing, technology-driven solutions for prevention and chronic disease management
4. Training and technical assistance for the adoption of technology, data, and innovative care models
5. Recruitment and retention of the rural health workforce
6. Innovative care models and value-based care arrangements
7. Right-sizing of healthcare delivery systems
8. Capital expenditures for existing rural facilities (with material limitations — see 2.5)

2.5 Notable Funding Restrictions

CMS guidance and state-level implementation documents (e.g., Alaska DOH) have clarified the following:

- **No new construction, major building expansion, or land acquisition.** Capital use is limited to minor alterations or renovations of existing facilities.
- **No supplanting of Medicaid revenue.** Funds cannot be used to replace operating revenue lost due to other provisions of P.L. 119-21.
- **Cost allocation required for dual-use infrastructure.** For example, if connectivity serves both RHTP and general operations, only the RHTP-attributable share is allowable.

These restrictions steer state spending away from real estate and toward technology, workforce, and service-model investments.

3. State Distribution Archetypes

States are distributing RHTP funds through three broad mechanisms. Many states use hybrids, but one archetype typically dominates.

Archetype 1 — Centralized Procurement

State agency retains funds and issues enterprise-scale RFPs for statewide systems (examples: Rhode Island's exploration of a state-sponsored EHR; Vermont's shared-services network; Indiana's centralized Medical Operations Coordination Center). Procurement decisions are made at the state CIO/CTO or Medicaid agency level.

Archetype 2 — Regional Coalition Pass-Through

State divides funds among regional non-profits, hospital collaboratives, or multi-county coalitions (examples: Pennsylvania's Rural Care Collaboratives; Indiana's eight-region model for its 60% regional allocation; North Carolina's NC ROOTS Hubs). Procurement is made by the lead agency of each regional coalition.

Archetype 3 — Competitive Sub-Grant Portal

State acts as pass-through, managing an application portal for many smaller grants to individual clinics and providers (examples: Oklahoma's microgrant program with awards of $50K–$250K; South Carolina's Connections to Care; New Jersey's four-activity RFA structure). Procurement is distributed across many small buyers.

4. State-by-State Analysis

Every state's plan is drawn from published CMS state project abstracts, state agency websites, or state press releases. Where a plan detail is not explicitly confirmed in a published state document, the text notes this and labels the item as inferred.

Per CMS state project abstracts and the Northwest Regional Telehealth Resource Center (NRTRC) summary, the Year-1 awards and lead agencies are as follows:

4.1 Northeast Region

Connecticut — FY2026 Award: $154,249,106 | Lead Agency: Department of Social Services (DSS)

CT is one of the few states where the Medicaid agency leads rather than DOH. DSS will coordinate with the Office of Rural Health, Office of Health Strategy, and Department of Mental Health and Addiction Services. The state application identifies 31 population-health projects focused on treatment barriers and patient-centered care. Explicit priorities include interoperability and analytics, which points toward enterprise data integration work rather than device-level procurement.

Delaware — FY2026 Award: $157,394,964 | Lead Agency: Department of Health and Social Services (DHSS)

Delaware's plan consolidates spending into approximately 15 larger initiatives rather than small grants. Published allocations include approximately $97.5M for "Hope Centers" (integrated service sites) and a multi-year medical education investment with rural training pipeline goals. A significant Health Information Exchange allocation is also included. Specific figures should be verified against the Delaware state project narrative before use in external materials.

Maine — FY2026 Award: $190,008,051 | Lead Agency: Department of Health and Human Services (DHHS)

Maine's plan includes a "Rural AI Hub" concept for centralized analytics services to rural providers and capital grants to retrofit rural hospitals for Program of All-Inclusive Care for the Elderly (PACE) models. The Rural AI Hub's eventual hosting model — academic partner, state-hosted, or commercial cloud — is not yet specified in publicly available documents.

Massachusetts — FY2026 Award: $162,005,238 | Lead Agency: Executive Office of Health and Human Services (EOHHS)

Massachusetts, which has a limited rural footprint, concentrates its plan on social drivers of health, Tribal partnerships (Mashpee Wampanoag and Aquinnah Wampanoag), and workforce housing support. The plan also includes an initiative (sometimes referenced internally as "SHINE HT") to help community health centers adopt technology solutions within their EHR systems.

Note: *This is distinct from the federal SHINE Medicare insurance counseling program, despite naming overlap.*

New Hampshire — FY2026 Award: ~$150,000,000 | Lead Agency: Department of Health and Human Services (DHHS)

NH prioritizes the Certified Community Behavioral Health Clinic (CCBHC) model, with funds flowing to Community Mental Health Centers for IT upgrades required to meet CCBHC federal reporting standards. The plan also establishes a family medicine residency in Coos County, likely partnered with Androscoggin Valley Hospital.

New Jersey — FY2026 Award: $147,250,806 | Lead Agencies: Department of Health (NJDOH) and Department of Human Services (NJDHS), co-leading; distribution managed through NJDOH Office of Primary Care and Rural Health

NJ received the smallest total award but one of the highest per-rural-resident allocations. The state has issued the "Rural Health Transformation Program 2026: Advancing Technology, Prevention, and Workforce Capacity" RFA, structured around four activities:

- **Activity A — Rural Preventive Health Initiatives** (early detection, health education, chronic disease interventions, with emphasis on uninsured/underinsured populations)
- **Activity B — Technology Solutions, Telehealth, and Remote Patient Monitoring** (telehealth expansion, digital tools, RPM)
- **Activity C — Training and Capacity Building** (cultural competency, workforce training)
- **Activity D — Clinical Workforce Recruitment and Retention Incentive** (limited to one awardee)

An initial sub-award round totaling approximately $28M across the four activities is under review. The state has published an RHTP Vendor Directory; inclusion does not constitute endorsement. NJ has no federally recognized tribes.

New York — FY2026 Award: $212,058,208 | Lead Agency: Department of Health (DOH)

NY's plan centers on the "Rural Roots" workforce pipeline (flowing to SUNY institutions and partner health systems), eConsult partnerships with academic medical centers (e.g., URMC, Bassett Healthcare), and an explicit cybersecurity line item for rural facilities.

Pennsylvania — FY2026 Award: $193,294,054 | Lead Agency: Department of Human Services (DHS)

Pennsylvania is standing up regional "Rural Care Collaboratives" expected to be staffed and meeting by end of 2026. Priorities include tele-EMS (tele-stroke, tele-behavioral health in ambulances) and linking rural hospitals to referral centers such as UPMC and Geisinger.

Rhode Island — FY2026 Award: $156,169,931 | Lead Agency: Executive Office of Health and Human Services (EOHHS)

With the highest per-rural-resident award in the country (~$6,305), RI is using funds for concentrated pilots. The state application references a state-sponsored EHR concept and a centralized rural health data hub. RI has established a Rural Stakeholder Advisory Committee, which has designated the Rhode Island Health Center Association as the oversight body for community health center allocations.

Vermont — FY2026 Award: $195,053,740 | Lead Agency: Agency of Human Services (AHS)

VT's "Clinically Integrated Network of Shared Services" funds a shared EHR for participating providers, a statewide closed-loop referral platform, and workforce housing. Vermont's plan leans more than most states toward enterprise shared-services procurement.

4.2 Southeast Region

Alabama — FY2026 Award: $203,404,327 | Lead Agency: Alabama Department of Economic and Community Affairs (ADECA), partnering with Finance and Medicaid

Alabama is building a new School of Healthcare Sciences to anchor rural workforce training. Plan elements include regional shared-service IT hubs for clusters of hospitals and digital obstetric regionalization, which includes telerobotic ultrasound in the state narrative.

Arkansas — FY2026 Award: $208,779,396 | Lead Agency: Department of Finance and Administration (DFA)

Arkansas frames its plan around three initiatives: HEART (nutrition / Farm-to-School), PACT (Promoting Access — funding flows to Clinically Integrated Networks and hospitals for facility upgrades and M&A stabilization), and THRIVE (digital infrastructure, including a referenced "ROADMAP" data system).

Florida — FY2026 Award: $209,938,195 | Lead Agency: Agency for Health Care Administration (AHCA)

FL's plan contains approximately 15 distinct initiatives, using a competitive RFA approach. Specific allocations include approximately $5M for Health Information Exchange / Encounter Notification Service onboarding and a 10% cap on Tech Catalyst funding. Renovations are capped at 20% of spending, consistent with federal capital-use restrictions.

Georgia — FY2026 Award: $218,000,000 (approximate, per state sources) | Lead Agency: Department of Community Health (DCH)

The state's "GREAT Health" program includes treat-vs-transport telehealth pilots for EMS and deployment of obstetric telehealth carts in rural EDs. GA is also positioning RHTP spending to align with the CMS AHEAD (States Advancing All-Payer Health Equity Approaches and Development) model.

Kentucky — FY2026 Award: approximately $200,000,000 (published CMS figures should be confirmed) | Lead Agency: Cabinet for Health and Family Services (CHFS)

KY is establishing an "A3 Team" within the Department for Public Health to coordinate initiatives. Priorities include maternal health in maternal-care deserts, the EmPATH psychiatric emergency model, and FQHC-based chronic care hubs.

Louisiana — FY2026 Award: $208,374,448 | Lead Agency: Louisiana Department of Health (LDH)

LDH's plan includes a Rural Tech Catalyst Fund (RTCF-LA) — a dedicated allocation for rural provider technology acquisition — alongside "Rural HERO" workforce incentives and tele-EMS pilots.

Mississippi — FY2026 Award: $205,907,220 | Lead Agency: State Department of Health (MSDH)

MSDH frames its plan as "BRIDGE" (Building Rural Infrastructure for Delivery, Growth and Efficiency), with a "Health Technology Advancement and Modernization" (HTAM) line item funding telehealth and health IT upgrades, plus capital investments in psychiatric emergency services.

North Carolina — FY2026 Award: $213,008,356 | Lead Agency: NCDHHS, Office of Rural Health

NCDHHS is launching regional "NC ROOTS" Hubs connecting medical, behavioral, and social supports. The Duke-Margolis Center for Health Policy is named as a subrecipient for policy design and evaluation. A central theme is supporting rural providers through value-based care transitions.

South Carolina — FY2026 Award: $200,030,252 | Lead Agency: South Carolina Department of Health and Human Services (SCDHHS / Medicaid)

SC's "Connections to Care" initiative expands digital infrastructure through EHR, RPM, and a statewide resource database. A "Tech Catalyst Fund" provides seed funding for rural health technology startups — an unusual state-level posture that functions as venture capital for rural health tech.

Tennessee — FY2026 Award: $206,888,882 | Lead Agency: Department of Health (TDH)

TN's plan is the most complex, with 17 distinct initiatives under the "MaRTHA" (Make Rural TN Healthy Again) banner. Notable technology elements include "TN Community Compass" (resource referral) and a statewide eConsult platform.

Virginia — FY2026 Award: $189,544,888 | Lead Agency: Department of Medical Assistance Services (DMAS / Medicaid)

DMAS is hiring a core team in 2026 to manage "VA Rural Vitality." Priorities include a tribal grants program (Virginia has federally recognized tribes), OB/GYN shortage mitigation, and broadband/telehealth infrastructure.

West Virginia — FY2026 Award: $199,476,099 | Lead Agency: Department of Health and Human Resources (DHHR)

WV's "Mountain State Care Force" includes return-to-home workforce incentives, a "Connected Care Grid" for on-demand virtual care, and a "Smart Care Catalyst" grant program for tech-enabled innovation.

4.3 Midwest Region

Illinois — FY2026 Award: $193,418,216 | Lead Agency: Department of Healthcare and Family Services (HFS)

HFS structures its plan in three categories: Transforming Rural Healthcare, Overcoming Geographic Barriers, and Building a Resilient Rural Workforce. HFS held 38+ stakeholder meetings during application development. Funding includes direct grants for service line conversion, virtual care platforms, and mobile units.

Indiana — FY2026 Award: $206,900,000 | Lead Agency: Family and Social Services Administration (FSSA) with Indiana Department of Health (IDOH)

IN publishes a defined 40/60 split: 40% for statewide initiatives, 60% for regional grants distributed across eight regions. The statewide Medical Operations Coordination Center (MOCC) is budgeted at approximately $56.2M and functions as a centralized patient-transfer coordination system — a significant single-contract opportunity.

Iowa — FY2026 Award: $209,040,064 | Lead Agency: Department of Health and Human Services

IA's "Hometown Connections" plan emphasizes tying local care to specialized centers and subsidizing HIE connections. The University of Iowa Holden Comprehensive Cancer Center is expected to play a role in oncology satellite networks, and CyncHealth is referenced as the state HIE entity for rural onboarding subsidies.

Kansas — FY2026 Award: $221,898,008 | Lead Agency: Kansas Department of Health and Environment (KDHE)

KDHE's plan targets 100% of rural Medicaid beneficiaries in Accountable Care Organizations by 2031, with funds supporting technical assistance, downside-risk protection, and RPM.

Michigan — FY2026 Award: $173,128,201 | Lead Agency: Department of Health and Human Services (MDHHS)

MI is expanding mobile crisis infrastructure through Community Mental Health Services Programs (CMHSPs) and listing tribal entities as direct subrecipients. Michigan's per-rural-resident figure is among the lowest nationally (~$26,611 per 1,000 rural residents per KFF).

Minnesota — FY2026 Award: $193,000,000+ | Lead Agency: Minnesota Department of Health (MDH)

MDH's "Collaboration for Financial Viability" program funds chronic-management technology and tribal Community Health Representative support. Grants to University of Minnesota and regional systems expand rural rotations.

Missouri — FY2026 Award: approximately $200,000,000 (verify against CMS project abstracts) | Lead Agency: Department of Health and Senior Services

Plan elements include local community hub coordination and a "Digital Backbone" state-level interoperability procurement.

Nebraska — FY2026 Award: $219,000,000 (approximate) | Lead Agency: Department of Health and Human Services (DHHS)

NE's plan includes a re-established "Presidential Fitness Test" (funds to school districts), "Whole foods through local growers" grants to agricultural cooperatives, and hospital billing-readiness support coordinated with the Nebraska Hospital Association.

North Dakota — FY2026 Award: $198,936,970 | Lead Agency: Department of Health & Human Services (HHS)

ND has a unique structure: RHTP funds are managed in partnership with the Bank of North Dakota, which administers a rural health medical facility infrastructure loan program providing short-term gap financing to grant recipients. Plan elements include "Connect Tech" ($168M over five years for EHR/payer hub modernization) and "Eat Well ND" nutrition programs.

Ohio — FY2026 Award: $202,030,262 | Lead Agency: Ohio Department of Health (ODH)

ODH uses tiered funding for regional Clinically Integrated Networks (CINs) to coordinate care, shared services, and telehealth. The "Ohio Rural Health Transformation Project" emphasizes maternal/infant vitality and substance use response.

South Dakota — FY2026 Award: $189,477,607 | Lead Agency: Department of Health

SD's "Tech and Data Connection for a Healthier South Dakota" modernizes digital health and launches a "Rural Data Atlas." A parallel Medicaid Primary Accountable Care Transformation moves rural providers from fee-for-service toward partial capitation. Tele-EMS partnerships (e.g., Avel eCare model) are referenced.

Wisconsin — FY2026 Award: $203,670,005 | Lead Agency: Department of Health Services (DHS)

WI allocates $27.5M to Tribal Nations. Additional elements include payment reform (cost-based to prospective payment for rural clinics) and $230M over five years in care coordination grants to regional networks.

4.4 Southwest and South-Central Region

Arizona — FY2026 Award: $166,988,956 | Lead Agency: Arizona Health Care Cost Containment System (AHCCCS / Medicaid)

AZ leverages managed-care organizations for distribution. Priorities include telehealth links between Phoenix/Tucson specialists and rural clinics, and workforce pipelines (Northern Arizona University is a likely academic partner based on state context).

New Mexico — FY2026 Award: $211,484,741 | Lead Agency: Human Services Department (HSD)

Specific plan details are less publicly documented than for other states at the time of this writing. Given state demographics, a significant portion is expected to pass through to Indian Health Service and tribal

638 facilities. Project ECHO (originated at the University of New Mexico) is a likely beneficiary for remote specialist consult expansion, though this is inferred rather than confirmed.

Oklahoma — FY2026 Award: $223,476,949 | Lead Agency: Oklahoma State Department of Health (OSDH)

OK has adopted a direct microgrants program with individual awards anticipated in the $50,000–$250,000 range, targeted at one-time equipment and technology purchases by rural clinics. This archetype calls for high-volume, channel-driven sales motions rather than enterprise contracts.

Texas — FY2026 Award: $281,319,361 | Lead Agency: Health and Human Services Commission (HHSC), with State Office of Rural Health

Texas captured the largest Year-1 award. Its "Rural Texas Strong" framework includes "Lone Star Advanced AI and Telehealth" and "Rural Texas Patients in the Driver's Seat." Initial state materials indicate approximately 163 rural hospitals surveyed, with priorities on cybersecurity hardening and statewide RPM networks. Despite the large total award, Texas's per-rural-resident figure is the lowest in the country (~$66).

4.5 Western Region

Alaska — FY2026 Award: $272,174,856 | Lead Agency: Alaska Department of Health (DOH)

Alaska is the second-largest recipient and has the highest per-1,000-rural-resident figure (~$662,755 per KFF). DOH has contracted with the Alaska Community Foundation (ACF) to manage the subaward process, application portal, and payment processing. Priorities include medical transport infrastructure (given the absence of road networks for many communities), tribal health organizations, and the "Spark Technology and Innovation" initiative focused on AI/wearables for village monitoring. Alaska's first budget period runs December 29, 2025 through October 30, 2026.

California — FY2026 Award: $233,639,308 | Lead Agency: Department of Health Care Access and Information (HCAI)

HCAI's plan follows a hub-and-spoke model anchored by larger hospitals. A new "Rural Technical Assistance Center" will support smaller providers on HIE and cybersecurity implementation, and a "Statewide Workforce Mapping Tool" procurement is included.

Colorado — FY2026 Award: $200,105,604 | Lead Agency: Department of Health Care Policy & Financing (HCPF / Medicaid)

HCPF uses funds to top up Medicaid rates or provide capital grants tied to the Hospital Transformation Program (HTP). Behavioral health integration into primary care and conditional solvency grants to independent rural hospitals are core elements.

Hawaii — FY2026 Award: $188,892,440 | Lead Agency: Executive Office / Department of Health

HI's plan emphasizes inter-island transport subsidies, neighbor-island residencies funded to John A. Burns School of Medicine (JABSOM), and integration of Native Hawaiian traditional healers.

Idaho — FY2026 Award: $185,974,368 | Lead Agency: Department of Health and Welfare (IDHW)

IDHW pursues data-driven, community-targeted right-sizing. Tech elements focus on tele-stroke and tele-psych for rural hospitals. A tribal set-aside of 3–10% is referenced in state materials.

Montana — FY2026 Award: $233,509,359 | Lead Agency: Department of Public Health and Human Services (DPHHS)

MT's plan prioritizes operational solvency for frontier Critical Access Hospitals and expanded micro-credential and apprenticeship pathways through state colleges.

Nevada — FY2026 Award: $179,931,608 | Lead Agency: Department of Health and Human Services (DHHS)

NV's plan includes an adapted "Teach Nevada" scholarship program for health disciplines flowing through Nevada System of Higher Education institutions and a tribal set-aside.

Oregon — FY2026 Award: $197,271,578 | Lead Agency: Oregon Health Authority (OHA)

OHA is phasing implementation: Phase 1 (2026–2027) for shovel-ready catalyst grants; Phase 2 for long-term shared infrastructure. Workforce loan repayment and telehealth investments are priorities.

Utah — FY2026 Award: $195,743,566 | Lead Agency: Department of Health and Human Services (DHHS)

UT combines a value-based payment track ("FAST"), food-infrastructure investments ("PATH"), and a re-established "Gold Medal Schools" program.

Washington — FY2026 Award: $181,257,515 | Lead Agency: Health Care Authority (HCA)

WA sets aside $20M annually for Tribal entities and invests in University of Washington and Washington State University medical schools. Distribution is a mix of direct awards and competitive procurements.

Wyoming — FY2026 Award: $205,004,743 | Lead Agency: Wyoming Department of Health (WDH)

WY's "Perpetuity Fund" is a trust structure with allocations approximately 40.7% to CAH basic incentives and 26.9% to EMS regionalization. Funds flow primarily to CAHs and EMS agencies.

5. Cross-Cutting Themes

5.1 Tribal Sovereignty and Direct Allocation

Multiple Western and Midwestern states (WA, OR, AK, NM, OK, WI, MI) are allocating funds directly to Tribal Health Organizations rather than channeling through state bureaucracies. Washington's $20M/year set-aside and Wisconsin's $27.5M allocation are the clearest examples. This gives THOs independent procurement authority, often on faster timelines than state agencies. Note that P.L. 119-21 did not include a statutory tribal set-aside; state-level decisions account for all tribal allocations.

5.2 State-Sponsored Technology Stacks

A subset of states is shifting from "give hospitals money to buy tech" toward "the state buys tech on behalf of hospitals." Examples include Vermont and Rhode Island exploring shared-EHR instances where the state holds the license, Indiana's centralized MOCC, and Texas's and Maine's state-run AI initiatives. This shift favors enterprise-scale procurements over fragmented hospital-by-hospital sales.

5.3 Workforce as Data Infrastructure

California, New York, and Delaware have framed workforce as a data problem rather than only a scholarship problem. Workforce mapping tools, licensure databases, and pipeline metrics systems are appearing as distinct procurement categories.

5.4 Microgrant and Catalyst Portals

Oklahoma, Oregon, Louisiana, and several others have stood up microgrant or "catalyst" portals with relatively small awards (typically $50K–$250K) for shovel-ready projects. This archetype supports a high-velocity, channel-based sales motion rather than direct enterprise engagement.

5.5 Behavioral Health and CCBHC Expansion

Multiple states (NH, MI, IL, KY, OH) are routing significant funds to Community Mental Health Centers and CCBHC buildouts. CCBHCs have heavy federal reporting requirements, which creates demand for EHR upgrades, reporting platforms, and analytics capabilities specific to behavioral health.

5.6 Tele-EMS and Pre-Hospital Care

Pennsylvania, Georgia, South Dakota, Louisiana, and others are equipping EMS for tele-stroke, tele-behavioral health, and treat-in-place models. This is driving demand for ruggedized tablets, cellular routers, and mobile connectivity platforms.

6. Relevance for Dell Planning

The following observations are provided as market context, not as specific sales recommendations.

Procurement decisions are moving up the chain. In most states, the effective customer for RHTP-funded technology is a state agency, regional coalition lead, or newly formed statewide innovation hub — not an individual rural hospital. Account teams engaging only at the hospital level may miss the decision point.

Archetype matters more than geography. A state's archetype (centralized, regional coalition, microgrant portal) is a stronger predictor of appropriate sales motion than its census region. Two neighboring states can require completely different engagement models.

Technology categories likely to see direct RHTP-funded demand:

- Shared-service hubs and cloud infrastructure for multi-hospital EHR / cybersecurity hosting
- RPM devices, platforms, and associated data infrastructure
- Telehealth endpoints (carts, mobile units, EMS tele-equipment)
- Cybersecurity stacks for rural hospitals, particularly in states that named cybersecurity as an explicit line item (NY, TX, CA)
- Analytics and interoperability platforms at the state HIE level
- Learning management and simulation systems tied to workforce initiatives
- Workforce data platforms (licensure, vacancy, pipeline tracking)

Timeline sensitivities. Per NASHP, states have until September 30, 2027 to obligate Year-1 funds. Most states are still in procurement-setup mode in early-to-mid 2026. Engagement timing should be calibrated to each state's procurement cycle.

Caveats to underscore internally.

- RHTP is a cooperative agreement with significant CMS oversight. Sub-award structures can be modified by states (with CMS approval) throughout the program period.
- Award amounts reported for states like Kentucky, Missouri, Nebraska, and Delaware should be confirmed against CMS state project abstracts before being cited externally.

- RHTP does not replace Medicaid revenue losses under P.L. 119-21. Rural providers will face net financial pressure even with RHTP funds, which may affect their ability to fund matching investments or operating costs.

7. Sources

#	Source
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